

Guide To Initiating Hormone Replacement Therapy (HRT)

Oestrogen replacement for low oestrogen levels (treats symptoms)
 Progesterone for endometrial protection (if uterus intact)
 + Contraception for 1yr if LMP >50yo
 + Contraception for 2yrs if LMP <50yo

Perimenopause, still having periods, LMP < 1yr ago, age under 53 or Premature Ovarian Insufficiency

Combined Sequential HRT = Oestrogen daily, Progesterone 14 days per month or Mirena Coil

Post Menopause, LMP>1yr ago

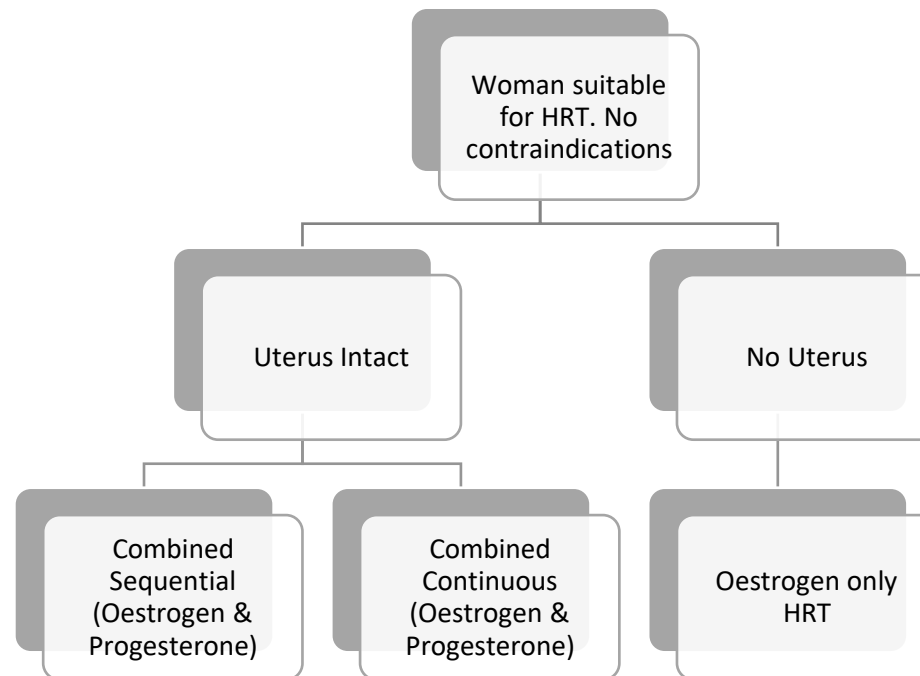
Combined Continuous HRT = Oestrogen & Progesterone daily OR Oestrogen daily & Mirena Coil

Assessment:

Symptoms
 Main symptom of concern
 Impact on quality of life
 What has been tried & outcome
 Risk factors: personal & family history of Breast Cancer & Thromboembolism
 Osteoporosis risk assessment

Resources

[Easy HRT Prescribing Guide](#)
[Top 10 Tips for HRT Prescribing](#)
[BMS Menopause Videos](#)
[BMS HRT Risk Chart](#)
[FRAXX Tool](#)



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Approved by North Middlesex University Hospital, University College London Hospital & Whittington Hospital Gynaecologists

Based on most recent published guidelines

March 2021, Review March 2026

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[Alternatives during Shortages](#)

Combined Sequential HRT

Oral

Femoston® 1/10 ↑ to 2/10
Elleste Duet® 1mg ↑ to 2mg

Patches

Evorel Sequi®
Femseven Sequi®

Combined Continuous HRT

Oral

Femoston Conti®
Elleste Duet Conti®

Patches

Evorel Conti®
Femseven Conti®

Separate Sequential HRT

Oestrogen Daily

Oral: Estradiol 1mg ↑ to 2mg
Patch: Estradiol 25mcg ↑ in 25mcg increments. *Change twice a week.*
Gel: Oestrogel® 1-2 pumps daily ↑ up to 4 pumps max.

Progesterone D14 to 28 each month

Oral: Micronised Progesterone 200mgs nocte or Utrogestan® (off license use vaginally 14 nights) *Improves sleep*
Oral: Medroxyprogesterone 10mgs od
Mirena® coil: 4yrs (5yrs FSRH)

Separate Combined HRT

Oestrogen Daily

Oral: Estradiol 1mg ↑ to 2mg
Patch: Estradiol 25mcg ↑ in 25mcg increments. *Change twice a week.*
Gel: Oestrogel® 1-2 pumps daily ↑ up to 4 pumps max.

Progesterone Daily

Oral: Micronised Progesterone 100mgs nocte or Utrogestan® (off license 200mgs alt nights vaginally)
Oral: Medroxyprogesterone 5mgs od
Mirena® coil: 4yrs (5yrs FSRH)

Vaginal Atrophy

Vaginal oestrogen used alone or with HRT
Ovestin® cream 2x per week
Vagifem® up to 5x per week
Estring® changed every 3 months
Non-Hormonal options:
Vaginal moisturiser twice a week
Vaginal Lubricant for intercourse

Transdermal has No VTE risk; good for smokers, Obesity, high BP, migraine
Regular withdrawal bleeds with Sequential HRT. May have irregular bleeding. Review if heavy bleeding. If bleeding settles on cessation of HRT, consider alternative preparation.
Review 3m, 6m, then annually
Document risks- VTE (with oral), breast cancer 5/1000 extra cases, irregular bleeding
[BMS HRT Risk Chart](#)
Document benefits-symptom control, reduce fragility fractures

Red Flags:
Post-Menopausal bleeding despite cessation of HRT



2ww Referral

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